

Our Ref: (T) mb/ss/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

CG89, G level, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses :

Crohn's disease

Ileocolonic resection

Medications :

Humira 40 milligrams every other week (January 2017)

Azathioprine 50 milligrams daily (currently on hold)

Previous Medication: Infliximab loss of response.

I reviewed [NAME] in clinic today together with her parents. It has been some time since we f reviewed [NAME]. Back in January she underwent colonoscopy which shows disease recurrence at anastomosis. At that time we have made the decision to discontinue Infliximab infusion and sw Humira. Unfortunately she had failed to attend a couple of appointments and had not had any u blood monitoring I therefore had to put her Humira account on holds. It has been a couple of she has last had her Humira and has noticed a significant effect with not having it. Abdomina slight increase in frequency as her main concern over the past couple of days. She has respon extremely well to the medication and it is clear that we need to continue and I have stressed importance of having a periodic review and regular blood monitoring.

Her fatigue is an ongoing issue and she is struggling with this. It may be related to her aza and in view of repeated viral infection I have suggested that she hold off taken the azathiop month or so to see if this improved her symptoms in anyway. If it does not then I have stress [NAME] that she will need to restart taken. Meanwhile she will continue on Humira every other suspect this is going to need to be for the long term. [NAME] is also struggling with slight phobia and she was asking if anybody else was able to administer her injection. I think she d her boyfriend on one occasion how this lead to arguments and I have suggested that her boyfri the ideal person to be administering her injection. I have asked her if she can approach the nurses and your surgery to see if they are able to administer her injections once a fortnight couple of injections just to see if this does decrease her anxiety. I am more than happy to f copy of her Humira prescription if required. Failing that I will ask the healthcare home team a nurse visit for one or two of the injections. Past experience shows they taken the pressure administering the injections over the next few weeks usually reduce their anxiety of the need subside. I very much hope you are able to support [NAME] with this.

Meanwhile she informs me that there is a possibility of her move to Spain at which point we w obviously have to work out the logistics of her having regular blood monitoring an indeed get to the medication. She is going to keep me updated with her plan. Meanwhile we will see her f in clinic for a year's time and she has my secretary's contact number should she run into any

With kind regards

Yours sincerely

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Sister [NAME]c (Hons)

Biologics CNS Independent Nurse [NAME]